

Swallowing Difficulty - Female (Child)

Algorithm 1336 | Child (0-17 years) | Gender at birth: Female

Definition

- UAT-only adult and pediatric dysphagia pathway separating airway obstruction, aspiration and acute neurological emergencies from swallowing difficulty requiring prompt in-person assessment.
- Child patient.
- Female.

Initial assessment

1. [DURATION] When did swallowing change: suddenly today or gradually, and is it worsening?
2. [OPEN_TEXT] Can the person swallow saliva, water, medicines, soft food and solids; where does it feel stuck; and is swallowing painful?
3. [YES_NO] Is there choking, ineffective or silent cough, breathing difficulty/noisy breathing, drooling, wet/gurgly voice, food or drink returning through the nose, or shortness of breath after swallowing?
4. [YES_NO] Did symptoms start with facial droop, arm/leg weakness or numbness, speech change, confusion, severe headache, collapse, or another possible stroke sign?
5. [YES_NO] Is there sudden tongue/lip/throat swelling, hives, wheeze, voice change, a suspected bone/foreign body, chemical ingestion, neck trauma, or caustic/hot-liquid burn?
6. [OPEN_TEXT] What is the age, underlying neurological/developmental condition, recent surgery/intubation, chest infection history, weight loss/dehydration, and usual prescribed swallowing or feeding plan?
7. [YES_NO] For a baby or child, is there poor feeding, cyanosis, sweating or breathing change with feeds, fewer wet nappies, lethargy, recurrent vomiting, poor growth, or caregiver concern about aspiration?
8. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Swallowed Foreign Body

Did the problem start after swallowing a solid object, button battery, magnet, or sharp item?

A suspected swallowed object belongs in the Swallowed Foreign Body pathway.

1. Disposition level 100

Is the airway blocked or breathing difficult/noisy; is coughing ineffective or the person unable to speak; can they not swallow saliva or are they drooling with a complete obstruction; is there acute stroke sign, reduced consciousness, cyanosis, or rapidly developing tongue/lip/throat swelling?

Airway obstruction, complete oesophageal obstruction with inability to handle secretions, anaphylaxis and acute neurological dysphagia are time-critical emergencies. Infants and children can deteriorate rapidly.

1. Disposition level 70

With no emergency feature, is there any new or worsening swallowing difficulty, pain, food sticking, cough/choke or breathlessness with intake, wet/gurgly voice, nasal regurgitation, recurrent chest infection, weight loss/dehydration, medicine-swallowing difficulty, or infant/child feeding concern?

NHS.UK advises urgent assessment for adult or child dysphagia. Aspiration can be silent, and safe texture, fluid and medicine decisions require clinical and swallowing assessment rather than telephone trial-and-error.

Advice

Qatar airway or swallowing emergency response

- Call Qatar emergency services on 999 now and do not allow self-driving. If choking but coughing effectively, encourage coughing. If unable to breathe, speak or cough effectively, follow the 999 call-taker's age-appropriate choking instructions; do not perform a blind finger sweep. Start CPR if unresponsive and not breathing normally. Give nothing by mouth for complete obstruction, reduced consciousness, acute stroke signs or severe aspiration risk. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening breathing; blue or grey colour; reduced consciousness; unable to swallow saliva.

Prompt in-person swallowing assessment

- Arrange prompt in-person assessment through an approved Qatar adult or pediatric pathway, with speech-and-language/swallowing, medical, ENT, gastrointestinal or neurological input as indicated. Until assessed, stop any item that causes cough, choke, wet voice or breathlessness; use only an existing clinician-prescribed texture/feeding plan. Do not invent a texture, add thickener, crush/open tablets or stop essential medicine without pharmacist/clinician instruction. Call 999 for airway difficulty, inability to swallow saliva, acute neurological signs or deterioration. Exact non-emergency destinations are local approval required.
- Call Back If: choking episode; cannot swallow saliva; breathing or voice change; fever or chest symptoms; dehydration; weight loss or child poor growth.

First aid

- Call Qatar emergency services on 999 now and do not allow self-driving. If choking but coughing effectively, encourage coughing. If unable to breathe, speak or cough effectively, follow the 999 call-taker's age-appropriate choking instructions; do not perform a blind finger sweep. Start CPR if unresponsive and not breathing normally. Give nothing by mouth for complete obstruction, reduced consciousness, acute stroke signs or severe aspiration risk. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (accessed 2026-07-25)
- NHS.UK, "Dysphagia (swallowing problems)", <https://www.nhs.uk/symptoms/swallowing-problems-dysphagia/> (accessed 2026-07-25)

- American Speech-Language-Hearing Association, "Adult Dysphagia" and "Aerodigestive Disorders" (accessed 2026-07-25)

Provenance

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